PREOPERATIVE DIAGNOSIS: Pregnancy at 39 weeks 2 days gestation; prior low-transverse Cesarean delivery; patient desires repeat Cesarean section.

POSTOPERATIVE DIAGNOSIS: Term pregnancy delivered via uncomplicated repeat low-transverse Cesarean delivery of a vigorous male infant.

PROCEDURE: Repeat low-transverse cervical Cesarean section, bilateral tubal ligation by Parkland method.

SURGICAL TECHNIQUE: The patient was positioned in the left lateral tilt position on the operating table. Regional spinal anesthesia was administered with excellent sensory block to the T4 dermatome. The abdomen was prepped with chlorhexidine and draped in a sterile obstetric fashion. A Pfannenstiel skin incision was made through the prior mature scar, excising old keloid tissue. The incision was carried down through the subcutaneous tissue to the anterior rectus fascia, which was incised transversely and dissected bluntly off the underlying rectus abdominis muscles.

The peritoneal cavity was entered sharply under direct visualization. A bladder blade was positioned to expose the lower uterine segment. The vesicouterine peritoneum was identified, incised, and the bladder flap gently developed bluntly and reflected inferiorly. A transverse hysterotomy incision was created in the lower uterine segment using a scalpel, and extended bluntly in a cephalad-caudad fashion. The amniotic membranes were ruptured, producing clear amniotic fluid.

The fetal vertex was gently elevated through the hysterotomy incision while fundal pressure was applied. A vigorous male infant was delivered at 09:42. The mouth and oropharynx were suctioned with a bulb syringe. The umbilical cord was doubly clamped and severed. Cord blood was collected for arterial blood gas analysis and Rh testing. The infant was handed over to the waiting neonatal resuscitation team. APGAR scores were 9 at 1 minute and 9 at 5 minutes; birth weight was 3,420 grams.

The placenta was removed spontaneously with gentle traction on the cord and concurrent fundal massage. Uterine massage was performed, and intravenous oxytocin (30 units in 500 mL lactated Ringer's) was infused. The uterus was exteriorized and wiped clean of clotted blood and membranes. The lower uterine segment was closed in two layers: the first layer using #1 absorbable polyglactin in a continuous locked running fashion, and the second layer in an imbricating Lembert fashion ensuring complete hemostasis.

Bilateral tubal ligation via the Parkland technique was executed: mid-ampullary segments of both fallopian tubes were elevated, window created in the avascular mesosalpinx, the tube doubly ligated with 2-0 chromic catgut, a 2 cm segment excised, and tubal lumina visualized bilaterally. The uterus was returned to the peritoneal cavity. The rectus fascia was closed with 0 PDS running suture, subcutaneous layer approximated with 3-0 Vicryl, and skin closed with a running subcuticular 4-0 Monocryl.